

HOUSE No. 2193

The Commonwealth of Massachusetts

PRESENTED BY:

Christine P. Barber and David M. Rogers

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to ensure efficient and effective implementation of the Roadmap for Behavioral Health Reform.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Christine P. Barber</i>	<i>34th Middlesex</i>	<i>1/17/2025</i>
<i>David M. Rogers</i>	<i>24th Middlesex</i>	<i>1/17/2025</i>
<i>James K. Hawkins</i>	<i>2nd Bristol</i>	<i>2/11/2025</i>
<i>Natalie M. Higgins</i>	<i>4th Worcester</i>	<i>3/11/2025</i>
<i>Mary S. Keefe</i>	<i>15th Worcester</i>	<i>3/4/2025</i>
<i>Samantha Montaño</i>	<i>15th Suffolk</i>	<i>3/4/2025</i>
<i>James J. O'Day</i>	<i>14th Worcester</i>	<i>4/1/2025</i>

HOUSE No. 2193

By Representatives Barber of Somerville and Rogers of Cambridge, a petition (accompanied by bill, House, No. 2193) of Christine P. Barber, David M. Rogers and others relative to the implementation of behavioral health reform. Mental Health, Substance Use and Recovery.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to ensure efficient and effective implementation of the Roadmap for Behavioral Health Reform.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 6A of the General Laws is hereby amended by inserting after
2 section 16FF

3 the following section:-

4 Section 16GG. (a) As used in this section the following words shall, unless the context
5 clearly requires otherwise, have the following meanings:-

6 “Behavioral health services”, the evaluation, diagnosis, treatment, care coordination,
7 management or peer support of patients with mental health, developmental or substance use
8 disorders, inclusive of medication management.

9 “Roadmap”, roadmap for behavioral health reform.

“Roadmap services”, shall include, but not be limited to, services provided by a behavioral health access line pursuant to section 2WWWW of chapter 29 of the General Laws, services provided by community behavioral health centers as defined in section 13D½ of chapter 118E of the General Laws, mobile crisis intervention for youth, mobile crisis intervention for adults, youth community crisis stabilization, adult community crisis stabilization and services provided by behavioral health urgent care providers.

“Secretary”, the secretary of health and human services.

(b) The secretary of health and human services shall coordinate all activities of the commonwealth to support the efficient and effective implementation of the roadmap for behavioral health reform. The secretary shall set goals and prepare a plan every other year for the commonwealth for implementation of roadmap services. The secretary, in consultation with the office of health equity established under section 16AA, shall fully integrate health equity principles and apply a health equity framework to all duties and obligations.

(c) The secretary will facilitate the coordination of all executive office, state agency, independent agency, state commissions and local and regional entity activities that support roadmap implementation in the commonwealth. The secretary shall:

(1) develop and implement comprehensive, biennial strategic plans to ensure efficient and effective implementation of the roadmap; the plans shall address opportunities and challenges, including but not limited to: (i) staffing; (ii) public and private sector financing; (iii) rate adequacy; (iv) roadmap services capacity; (v) linguistic and cultural competency of roadmap services delivery; and (vi) coordination across the executive office of health and human services and with other state and local agencies;

(2) align processes and procedures across the executive office of health and human services to ensure efficiencies in: (i) licensing, credentialing, certification, and other regulatory requirements; (ii) contracting; (iii) billing; and (iv) other relevant service delivery and payment requirements;

(3) issue cohesive service delivery and payment system guidance as applicable;

(4) identify and disseminate evidence-based or evidence-informed practices designed to advance health equity and trauma-informed care through roadmap services;

(5) explore steps to combine the behavioral health access line with the 988 Suicide and Crisis Lifeline into one number and entity in the commonwealth to reduce complexity for individuals and families;

(6) plan and implement campaigns to raise awareness about roadmap services to behavioral health stakeholders, community-based stakeholders, and individuals and families historically marginalized by race, ethnicity, gender identity, sexual identity, and other factors; and

(7) develop and implement biennial plans to gather feedback about roadmap services; solicit feedback from a diverse array of stakeholders including families members, people with lived experience, providers, health plans, state agencies, advocacy organizations, schools, law enforcement, and community-based organizations; prioritize response from: (i) people with lived experience, including youth and caregivers; (ii) individuals and family members from marginalized communities; and (iii) people that have and have not received roadmap services; ensure the plan includes both qualitative and quantitative elements and may include surveys and listening sessions with people with lived experience and family members.

(d) (1) The secretary shall oversee, in partnership with the secretary of the executive office of public safety and security, behavioral health crisis response planning and implementation for the commonwealth, including but not limited to: (i) collaboration across the executive office of health and human services, executive office of public safety and security, division of medical assistance and its contracted entities, the department of public health, public safety answering points, law enforcement, 988 Suicide and Crisis Lifeline centers, emergency medical services, community behavioral health centers, hospital emergency departments, behavioral health urgent care providers, and other entities; (ii) strategic planning; (iii) implementation and alignment across departments; (iv) data review; and (v) performance improvement.

(2) The secretary shall ensure the following services are reimbursed to cover the cost of reserve staff and bed capacity for timely response to routine and surge patient demand: (i) youth mobile crisis intervention; (ii) adult mobile crisis intervention; (iii) youth community crisis stabilization; (iv) adult community crisis stabilization services; and (v) behavioral health urgent care.

(3) The secretary, in conjunction with the secretary of the executive office of public safety and security, the commissioner of the department of mental health, and the commissioner of the department of public health, shall detail the legal and regulatory authority for law enforcement to drop off individuals experiencing behavioral health crisis at community behavioral health centers and shall outline protocols for such drop offs.

(4) The secretary, in conjunction with the assistant secretary of the division of medical assistance, the commissioner of the department of mental health, and the commissioner of the

department of public health, shall: (i) examine point of entry plans for community behavioral health centers to ensure they are relevant for drop offs of individuals in behavioral health crisis by emergency medical services providers; (ii) determine adequate reimbursement for community behavioral health centers to meet point of entry plan requirements; and (iii) modify regulations, standards, policies, plans, and rates to facilitate drop offs of individuals in behavioral health crisis at community behavioral health centers by emergency medical services providers.

(e) (1) The secretary shall develop and manage a centralized data dashboard to monitor utilization of roadmap services, inequities and disparities in access to behavioral health care, and timeliness of services.

(2) The secretary shall develop and make publicly available an initial data dashboard not later than 6 months from the effective date of this act. The data in said initial dashboard shall: (i) be limited to the data the behavioral health access line, community behavioral health centers, youth mobile crisis intervention, adult mobile crisis intervention, youth community crisis stabilization, adult community crisis stabilization, and behavioral health urgent care providers are required to report to the executive office of health and human services, the department of mental health, the department of public health, the division of medical assistance, or their contracted entities; (ii) shall include, but not be limited to, utilization, patient reported satisfaction, compliance with performance specifications, Enterprise Invoice/Service Management (EIM/ESM) data, Healthcare Effectiveness Data and Information Set (HEDIS®) data, other quality performance measure data, community-based evaluations, inpatient dispositions, response times, and patient outcomes, as applicable to each roadmap service; (iii) shall be updated quarterly; and (iv) shall be presented in a de-identified form.

(3) The secretary shall update the data elements in the centralized data dashboard at least once every 3 years. Updates shall be informed by feedback from roadmap services and other mental health and substance use providers, people with lived experience, family members, and other stakeholders, and best practices at the national level and in other states. The secretary shall prioritize data elements that reflect patient demographics including, but not limited to, age, race, ethnicity, gender identity, and sexual orientation to help identify and address disparities in access, quality of care, and outcomes. The secretary shall ensure the dashboard includes elements specific to the behavioral health crisis system including, but not limited to: (i) volume; (ii) patient demographics; (iii) location of services provided; (iv) response time; (v) disposition; (vi) nature of law enforcement engagement, if applicable; (vii) health, placement, and quality outcomes; (viii) complaint themes and resolution times; and (ix) nature of resolutions.

(4) The secretary shall ensure the data in the centralized data dashboard is: (i) made publicly available; (ii) de-identified; (iii) updated at least quarterly; and (iv) analyzed for trends, gaps in access, timeliness, quality, and equity, and areas for improvement.

(f) Annually, not later than July 1, the secretary shall report on progress, and the overall progress of the commonwealth, toward implementation of the roadmap for behavioral health reform using, when possible, quantifiable measures and comparative benchmarks, including a description of quantitative and qualitative metrics used to evaluate activities and outcomes. The report shall be filed with the governor, the clerks of the senate and house of representatives, the joint committee on health care financing, and the joint committee on mental health, substance use, and recovery. The report shall be posted on the official website of the commonwealth.

SECTION 2. Section 21A of chapter 12C of the General Law, as appearing in the 2022 Official Edition, is hereby amended by inserting after the first paragraph the following paragraph:-

Every 3 years the center shall conduct an analysis of the statewide, payor-agnostic community behavioral health crisis system as defined in section 2WWWW of chapter 29 of the General Laws. The analysis shall examine expenditures for services supported by the Behavioral Health Access and Crisis Intervention Trust Fund including, but not limited to, the behavioral health access line, crisis evaluation, crisis follow-up, youth community crisis stabilization, adult community crisis stabilization, and outpatient community behavioral health center services. The analysis shall document the expenditures for and the utilization of said services by payor. The first analysis shall be submitted not later than June 30, 2026 with the clerks of the senate and house of representatives, the joint committee on health care financing, and the joint committee on mental health, substance use, and recovery. The analysis shall be made public on the center's website.

SECTION 2. Chapter 6D of the General Laws is hereby amended by inserting after section 21 the following section:-

Section 22. Every 3 years, the commission, in collaboration with the executive office of health and human services and the center for health information and analysis, shall prepare a roadmap for behavioral health reform financing and sustainability report. The report shall analyze the financial stability of roadmap services including a behavioral health access line as referenced in section 2WWWW of chapter 29 of the General Laws, services provided by community behavioral health centers as defined in section 13D1/2 of chapter 118E of the

General Laws, mobile crisis intervention for youth, mobile crisis intervention for adults, youth community crisis stabilization, adult community crisis stabilization, and services provided by behavioral health urgent care providers. The report shall address opportunities and challenges, including but not limited to: (i) staffing; (ii) public and private sector financing; (iii) rate adequacy; (iv) roadmap services capacity; and (v) linguistic and cultural competency of roadmap services delivery. The report shall identify any statutory, regulatory, or operational factors that may impact the financial stability and sustainability of roadmap services and their ability to meet the mental health and substance use needs of people across the commonwealth. The first report shall be submitted not later than June 30, 2026 with the clerks of the senate and house of representatives, the joint committee on health care financing, and the joint committee on mental health, substance use, and recovery. The report shall be published on the commission's website.

SECTION 3. Section 21A of chapter 12C of the General Law, as appearing in the 2022 Official Edition, is hereby amended by inserting after the first paragraph the following paragraph:-

Every 3 years the center shall conduct an analysis of the statewide, payor-agnostic community behavioral health crisis system as defined in section 2WWWW of chapter 29 of the General Laws. The analysis shall examine expenditures for services supported by the Behavioral Health Access and Crisis Intervention Trust Fund including, but not limited to, the behavioral health access line, crisis evaluation, crisis follow-up, youth community crisis stabilization, adult community crisis stabilization, and outpatient community behavioral health center services. The analysis shall document the expenditures for and the utilization of said services by payor. The first analysis shall be submitted not later than June 30, 2026 with the clerks of the senate and house of representatives, the joint committee on health care financing,

164 and the joint committee on mental health, substance use, and recovery. The analysis shall be
165 made public on the center's website.